Throat & Mouth Examination — OSCE Checklist

Deakin MD2 · BB4 · ENT

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Inspect mouth → Tongue → Tonsils/pharynx → Palpate neck**. *Use a bright pen torch with the mouth wide open.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Ask about sore throat, swallowing, voice; remove dentures if present †
☐	Equipment: pen torch, tongue depressor, gloves

2. Examination

	Step	What to do / say	Abnormal
Oral cavity			
☐	Lips and mucosa	Ulcers, lesions, colour	Aphthous ulcers, candida, malignancy †
☐	Dentition	Comment on teeth and gums	Dental caries, abscess
☐	Tongue	Appearance; ask to raise tongue to check underneath if indicated	Lesions, floor-of-mouth swelling
Oropharynx			
☐	"Say ahh"	Mouth wide, tongue protruded	Palate rises symmetrically; uvula central †
☐	Tongue depressor	If tonsils not seen: press middle third of tongue, not the back †	
☐	Tonsils	Size, symmetry, inflammation, exudate	Tonsillitis; asymmetry = quinsy or malignancy †
☐	Pharynx	Swelling, inflammation, exudate, post-nasal drip	
Neck (if clinically indicated)			
☐	Lymph nodes	From behind: submental, submandibular, tonsillar, cervical chain, supraclavicular, pre/post-auricular, occipital	Infection, malignancy
☐	Parotids*	Palpate in front of ear and over angle of jaw	Parotitis, tumour

3. Closing

☐	Thank patient; dispose of tongue depressor; wash hands
☐	Present: "Good dentition; tongue and oral mucosa normal; tonsils symmetrical without erythema or exudate; uvula central; no cervical lymphadenopathy."
☐	Completion: "I would complete the ENT exam and consider a throat swab if indicated."

4. Quick reference †

- **Peritonsillar abscess (quinsy)**: unilateral tonsil swelling, uvula deviated away, trismus, "hot potato" voice, drooling — urgent ENT referral.
- **Airway red flags**: stridor, drooling, unable to swallow saliva, tripod posture — do not examine with a tongue depressor; call for senior help.
- Uvula deviates **away** from a CN X lesion (links to cranial nerve exam).